Cranial Nerve Examination — OSCE Checklist

Deakin MD2 · Neuro · Time: 8 min (+2 min reading)

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are usually verbalised in the OSCE ("I would also…"). Compare* **left vs right** *throughout and narrate aloud.*

1. Introduction (WIPER)

☐	**W**ash hands / hand hygiene
☐	**I**ntroduce self (name, role)
☐	**P**atient ID: name and DOB
☐	**E**xplain the exam and gain consent
☐	**P**osition: seated, eye level, ~1 m apart
☐	Ask about **pain** before starting
☐	Equipment ready: pen torch, Snellen, red pin, ophthalmoscope, cotton wool, neurotip, 512 Hz fork, tongue depressor

2. General inspection (end of bed, 10–15 s)

	Look for	Significance
☐	Well or unwell	Distress, drowsiness, airway
☐	Bedside clues	Walking aids, NG tube, thickened fluids
☐	Head shape	Craniotomy scar, shunt, Paget's
☐	Ptosis / proptosis, pupils, gaze	CN III, Horner's, thyroid eye disease
☐	Facial asymmetry	Droop, flat nasolabial fold (CN VII / stroke)
☐	Scars, cachexia, rashes, skin lesions	Parotid/neck scars, ear vesicles (Ramsay Hunt)
☐	Hearing aids, glasses	Baseline sensory deficit
☐	Gait and posture	Ataxia, hemiplegic gait

3. Cranial nerves I–XII

	Step	What to do / say	Abnormal
CN I — Olfactory			
☐	Smell*	"Any change in smell or taste?" Formal test one nostril at a time	Anosmia: URTI, trauma, Parkinson's
CN II — Optic			
☐	Visual acuity	Snellen at 6 m, glasses on, each eye covered; record e.g. 6/6	Pinhole if reduced
☐	Visual fields	1 m apart, cover opposite eyes, red pin from 4 quadrants	Bitemporal / homonymous hemianopia
☐	Fundoscopy*	Ophthalmoscope: disc, vessels, macula	Papilloedema, optic atrophy
CN II + III — Pupils			
☐	Inspect pupils	Size, shape, symmetry	Anisocoria
☐	Direct + consensual	Light from the side into each eye	Absent = afferent/efferent defect
☐	Swinging light	Swing torch between eyes	RAPD (Marcus Gunn)
☐	Accommodation	"Look far away, now at my finger"	Constrict + converge
CN III, IV, VI — Eye movements			
☐	H test	Head still, follow finger slowly in an H; ask about diplopia	III: down + out, ptosis · IV: vertical diplopia · VI: no abduction
☐	Nystagmus	Hold at extremes of gaze	Cerebellar / vestibular
CN V — Trigeminal			
☐	Sensation	Cotton wool (± neurotip) V1 forehead, V2 cheek, V3 jaw; eyes closed	Loss in a division
☐	Motor	Palpate masseter + temporalis on clenching; open jaw against resistance	Jaw deviates to weak side
☐	Corneal reflex*	Cotton wisp to cornea from the side	No blink (V afferent, VII efferent)
☐	Jaw jerk	Finger on chin, mouth slightly open, tap	Brisk = UMN
CN VII — Facial			
☐	Raise eyebrows	"Look up at the ceiling"	Forehead spared = UMN
☐	Eyes shut tight	"Don't let me open them"	Weak closure
☐	Show teeth	"Show me your teeth"	Asymmetric smile
☐	Puff cheeks	"Blow out your cheeks" — press gently	Air escapes on weak side
CN VIII — Vestibulocochlear			
☐	Whisper test	Rub opposite tragus; whisper a number (e.g. 68, 100) at arm's length	Can't repeat = hearing loss
☐	Rinne	512 Hz fork on mastoid, then beside ear: "Which is louder?"	BC > AC = conductive

	Step	What to do / say	Abnormal
☐	Weber	Fork on vertex: "Where do you hear it?"	Lateralises (see box)
CN IX, X — Glossopharyngeal, Vagus			
☐	Palate	"Say Ahh" — palate rises symmetrically; ask about swallow, voice, cough	Uvula deviates AWAY from lesion
☐	Gag reflex*	Verbalise	Absent: IX sensory / X motor
CN XI — Accessory			
☐	Shrug shoulders	Against resistance (trapezius)	Weak on lesion side
☐	Turn head	Against resistance; feel opposite SCM	Weak turn AWAY from lesion side
CN XII — Hypoglossal			
☐	Tongue at rest	Inspect in mouth for wasting, fasciculations	LMN lesion
☐	Protrude + side to side	"Stick out your tongue, move it side to side"	Deviates TOWARDS lesion

4. Closing and presenting

☐	Thank patient, reposition, wash hands
☐	Completion statement: "To complete, I would perform fundoscopy, corneal and gag reflexes, formal smell/taste, a full peripheral neuro exam and audiometry if indicated."
☐	Present: "I examined Mr X, a [age]-year-old… Cranial nerves I–XII were grossly intact. This is a normal cranial nerve examination." If abnormal, lead with the key finding + likely site of lesion.

5. Quick reference

Hearing	Rinne	Weber
Normal	AC > BC both ears	Central
Conductive	BC > AC affected ear	TO affected ear
Sensorineural	AC > BC (both reduced)	AWAY from affected ear

Facial palsy	UMN (stroke)	LMN (Bell's)
Forehead	Spared	Involved
Eye closure	Preserved	Weak, Bell's phenomenon
Lesion side	Contralateral	Ipsilateral

- **Uvula** deviates AWAY from the lesion; **tongue** points TOWARDS it.
- **Ptosis + small pupil** = Horner's · **ptosis + dilated pupil + down-and-out** = CN III (pupil involved: think PComm aneurysm).
- **RAPD:** affected pupil paradoxically dilates when light swings to it = optic nerve lesion.
- **Bulbar (LMN IX–XII):** wasted, fasciculating tongue (MND) · **Pseudobulbar (UMN):** spastic tongue, brisk jaw jerk, emotional lability.
- **CP angle lesion** (acoustic neuroma): V, VII, VIII together; lost corneal reflex is early.
- **Pitfalls:** not asking about pain/diplopia, moving finger too fast, patient moving head, not comparing sides, examining silently.